PORTFOLIO SAMPLE · TENDER RESEARCH

Tender Opportunity Mapping for a New UK Domiciliary Care Provider

MARKET	UK
SECTOR	Social care
DOCUMENT	Stage 1 opportunity mapping and priority summary
MY ROLE	Assessed realistic routes to contracts for a newly CQC-registered provider and prioritised them into an action plan.

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Client names, people, locations, reference numbers and pricing have been removed or replaced to protect client confidentiality. Content is otherwise as written for the client.

Provider profile: newly registered domiciliary care provider, current capacity 40–60 care hours per week, operating in one English region.

Overview

This review identifies the most realistic early-stage opportunities for a newly registered domiciliary care provider, prioritising routes that value flexibility, responsiveness and safe mobilisation over trading history or scale.

Priority 1: Act Now

Route	Why	Recommended action
Local authority spot purchasing	Brokerage and discharge teams are often the most accessible route for newly registered providers, and allow services to scale gradually.	Engage with adult social care brokerage; position availability for short-term, discharge-related and flexible packages.
Dynamic Purchasing Systems (domiciliary and discharge lots)	Open DPS routes are suitable for providers with CQC registration and demonstrable capacity, even at an early stage.	Prioritise DPS lots aligned to domiciliary care and discharge/step-down; monitor further competitions under existing registrations.
NHS discharge / step-down pathways	Hospital discharge and Home First-style pathways prioritise speed, safe transitions and short-term support.	Initiate engagement with local hospital discharge teams; position as a responsive provider for short-term packages.

Priority 2: Prepare (short term)

- Wider regional DPS opportunities requiring light preparation.
- NHS trusted provider or referral pathways that require initial engagement.
- Simple mobilisation and service summaries to support referrals.

Priority 3: Later

- Supported living pathways and frameworks.
- Larger frameworks requiring delivery history and higher capacity, to be progressed once delivery evidence and staffing depth increase.

Recommended next steps

- Focus immediate efforts on Priority 1 routes.
- Prepare documentation and engagement for Priority 2.
- Revisit Priority 3 once consistent delivery evidence is in place.